

Institute of Preventative Medicine

Consultation Notes — Email Thread

From: Dr. A. Johnson <a.johnson@iofpm.com>

To: Dr. V. Patel <v.patel@iofpm.com>

Subject: RE: Consultation — Jane Smith, DOB 15/03/1975

Date: Thursday 09 December 2021, 14:32

Hi Dr. Patel, Thank you for referring Ms. Jane Smith for consultation. I have reviewed her imaging reports (MRCP, MRI Head, MRA Head) dated 18/11/2021. **Key Findings Summary:** • Multiple gallstones in the gallbladder (largest 2.8 cm) • No evidence of common bile duct obstruction • 3 mm left MCA aneurysm at M1 segment • No acute intracranial findings **Clinical Assessment:** Ms. Smith presents with uncomplicated cholelithiasis and incidental small cerebral aneurysm. The aneurysm should be discussed with a neurovascular specialist for risk stratification and follow-up imaging protocol. Regarding the gallstones, whilst they are asymptomatic based on the clinical information provided, patients often benefit from elective cholecystectomy to prevent future biliary complications. **Recommendations:** 1. Refer to neurosurgery for aneurysm review and surveillance strategy 2. Consider elective cholecystectomy if patient is willing and fit for surgery 3. Follow-up imaging in 6-12 months to assess aneurysm stability 4. Patient education regarding warning signs Please let me know if you require any further clarification or if there are additional clinical details I should be aware of. Best regards, Dr. A. Johnson Consultant Radiologist GMC: 5009876

From: Dr. V. Patel <v.patel@iofpm.com>
To: Dr. A. Johnson <a.johnson@iofpm.com>
Subject: RE: Consultation — Jane Smith, DOB 15/03/1975
Date: Thursday 09 December 2021, 16:15

Dear Dr. Johnson, Thank you for the comprehensive assessment and detailed recommendations. Your summary is extremely helpful and aligns well with my clinical concerns. I agree that whilst the gallstones are currently asymptomatic, the patient would benefit from discussion regarding elective cholecystectomy to prevent potential future complications. She is otherwise fit and well. I will prioritize the neurosurgery referral for the aneurysm. The incidental nature of this finding is concerning, and I appreciate your recommendation for formal risk stratification. I will arrange follow-up imaging as suggested and counsel the patient regarding the surveillance plan. Thank you again for your expert input. I will be in touch once the patient has been reviewed by neurosurgery. Best regards, Dr. V. Patel MBBS BSc (Hons) MRCS DOHNS MRCGP FRSPH +44 7590001200 GMC: 6001234 Institute of Preventative Medicine Floor 5, The Amphenol Business Complex Thanet Way, London

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